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Dr. Childress Vita:

[Dr Childress Vita & Domains of Specialized Expertise \(https://drcachildress-consulting.com/wp-content/uploads/2019/11/2026-Dr-Childress-Domains-of-Specialized-Expertise-and-Vita-4-26.pdf\)](https://drcachildress-consulting.com/wp-content/uploads/2019/11/2026-Dr-Childress-Domains-of-Specialized-Expertise-and-Vita-4-26.pdf).

Diagnostic Questions to be Answered:

[Child Custody Conflict Diagnostic Questions \(https://drcachildress-consulting.com/wp-content/uploads/2019/11/Differential-Diagnostic-Questions-1-26.pdf\)](https://drcachildress-consulting.com/wp-content/uploads/2019/11/Differential-Diagnostic-Questions-1-26.pdf).

Professional Consultation

Catalogue of Concerns v6.0 – Childress & Tsou (2026).(<https://drcachildress-consulting.com/wp-content/uploads/2019/11/Catalogue-of-Concerns-v6.0-Childress-Tsou-2.pdf>)

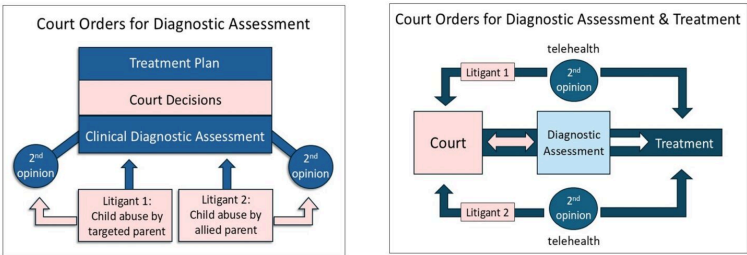
I am a court-involved clinical psychologist.

I provide consultation and expert testimony from clinical psychology surrounding the assessment, diagnosis, and treatment of court-involved child custody conflict. I am typically brought onto a matter in three roles:

General Consultation: to provide general consultation in clinical psychology to the attorney, client parent, and other court-involved professionals toward reaching an **accurate diagnosis** and effective treatment plan for the child and family.

Document Review: to review mental health and court-related documents provided by an attorney, and to provide a second opinion from the applied knowledge of clinical psychology to the information reviewed. I incorporate AI (Gemini-3) into my second-opinion document review.

Assessment Second Opinion: to participate in a court ordered assessment through telehealth as a second opinion consultant.



Treatment vs Custody

The focus of clinical psychology is treatment not custody The solution from clinical psychology is through diagnosis and treatment.

In the absence of child abuse, parents have the right to parent according to their cultural values, their personal values, and their religious values.

In the absence of child abuse, each parent should have as much time and involvement with the child as possible.

In the absence of child abuse, to restrict either parent’s time and involvement with the child would damage the child’s attachment bond to that parent, thereby harming the child and harming that parent.

Attachment Pathology

A child rejecting a parent is an attachment pathology, i.e., a problem in love and bonding. The only cause of a child rejecting a parent (disorganized attachment, i.e., no organized strategy to bond to the parent) is child abuse by one parent or the other.

Targeted Parent Abusive: either the targeted parent is abusing the child in some way, thereby creating the child's attachment pathology toward that parent,

Allied Parent Abusive: or the allied parent is psychologically abusing the child by creating a shared (induced) persecutory delusion and false (factitious) attachment pathology in the child for secondary gain to the parent.

I discuss diagnosing attachment pathology in the family courts in an online seminar on my YouTube channel:



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Persecutory Delusion

The pathology of concern is a possible persecutory delusion in the allied parent being imposed on the child, i.e., a shared (induced) persecutory delusion. I am a cited expert in the diagnostic assessment of delusional thought disorders (Walters & Friedlander, 2016; *Family Court Review*).

From Walters & Friedlander: “In some RRD families [resist-refuse dynamic], a parent’s underlying encapsulated delusion about the other parent is at the root of the intractability (cf. Johnston & Campbell, 1988, p. 53ff; **Childress**, 2013). An encapsulated delusion is a fixed, circumscribed belief that persists over time and is not altered by evidence of the inaccuracy of the belief.” (Walters & Friedlander, 2016, p. 426)

From Walters & Friedlander: “When alienation is the predominant factor in the RRD [resist-refuse dynamic}, the theme of the favored parent’s fixed delusion often is that the rejected parent is sexually, physically, and/or emotionally abusing the child. The child may

come to share the parent’s encapsulated delusion and to regard the beliefs as his/her own (cf. **Childress**, 2013).” (Walters & Friedlander, 2016, p. 426)

Walters, M. G., & Friedlander, S. (2016). When a child rejects a parent: Working with the intractable resist/refuse dynamic. *Family Court Review*, 54(3), 424–445. <https://doi.org/10.1111/fcre.12238> (<https://doi.org/10.1111/fcre.12238>).

I discuss diagnosing a persecutory delusion in an online seminar on my YouTube channel:



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Factitious Disorder Imposed on Another

The pathology of concern is a possible false (factitious) attachment pathology imposed on the child for secondary gain to the allied parent. The potential secondary gain from inducing false pathology in the child includes:

- Manipulating the Court:** to manipulate the Court’s decisions surrounding child custody,
- Spousal Abuse:** to emotionally and psychologically abuse the targeted parent using the child as the spousal abuse weapon,
- Regulatory Object:** to use the child as a regulatory object to meet the parent’s own emotional and psychological needs.

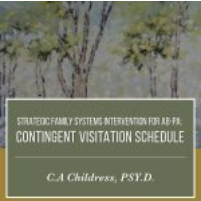
I discuss diagnosing a factitious disorder imposed on the child in an online seminar on my YouTube channel:



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A Contingent Visitation Schedule

[Contingent Visitation Schedule \(https://www.amazon.com/Strategic-Family-Systems-Intervention-AB-PA/dp/0996114556\)](https://www.amazon.com/Strategic-Family-Systems-Intervention-AB-PA/dp/0996114556): This 50-page



booklet represents a treatment protocol for resolving the family conflict when the diagnosis is a factitious

attachment pathology imposed on the child (DSM-5 300.19) and psychological child abuse (V995.51) by the allied parent.

In 2024 I presented a seminar on the Contingent Visitation Schedule to the national convention of the American Psychological Association. I then presented the exact same seminar to my YouTube Channel:

APA 2024: How to Use and Administer a Contingent Visitation Schedule in
Dr. C. A. Childress



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Books & Booklets

Foundations: An Attachment-Based Model of Parental Alienation
(<https://www.amazon.com/Attachment-Based-Model-Parental-Alienation-Foundations/dp/0996114505>).
(<https://www.amazon.com/Attachment-Based-Model-Parental-Alienation-Foundations/dp/0996114505>): In *Foundations* I describe the pathology in the family courts from three domains of established knowledge, family systems, personality disorder pathology, and attachment.

The Narcissistic Parent (<https://www.amazon.com/Narcissistic-Parent-Guidebook-Professionals-High-Conflict/dp/0996114548>).
(<https://www.amazon.com/Narcissistic-Parent-Guidebook-Professionals-High-Conflict/dp/0996114548>): This 50-page booklet is for legal professionals in the family courts and describes narcissistic personality pathology as it impacts parenting using direct quotes from the professional literature regarding various personality pathology features.

The Assessment of Attachment-Related Pathology Surrounding Divorce (<https://www.amazon.com/Assessment-Attachment-Related-Pathology-Surrounding-Divorce/dp/0996114572>).
(<https://www.amazon.com/Assessment-Attachment-Related-Pathology-Surrounding-Divorce/dp/0996114572>): This 50-page booklet presents a six-session assessment protocol for the child's attachment pathology and the family conflict.

Dark Personalities & Delusion Articles (Greenham & Childress)

Dark Personalities & Delusions I: Solving the Gordian Knot of Conflict in the Family Courts (https://www.researchgate.net/publication/369741224_Dark_Personalities_and_Induced_Delusional_Disorder_in_the_Family_Courts_Greenham_and_Childress_2023; ResearchGate).

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Dark Personalities and Induced Delusional Disorder, Part II: The Research Gap Underlying a Crisis in the Family and Domestic Violence Courts
(https://www.researchgate.net/publication/363197057_Dark_Personalities_and_Induced_Delusional_Disorder_in_the_Family_and_Domestic_Violence_Courts_Greenham_and_Childress_2023)



www.researchgate.net/publication/369741224_Dark_Personalities_and_Induced_Delusional_Disorders (2023; ResearchGate).

[Dark Personalities and Delusions III: Identifying Pathogenic](#)

www.researchgate.net/publication/368330924_Dark_Personalities_and_Induced_Delusional_Disorders

www.researchgate.net/publication/369741224_Dark_Personalities_and_Induced_Delusional_Disorders (2023; ResearchGate).

Quote Handout: Diagnosis

These are selected quotes from the report, [Improving Diagnosis in Healthcare](https://www.ncbi.nlm.nih.gov/books/NBK338596/) (<https://www.ncbi.nlm.nih.gov/books/NBK338596/>), that describe the diagnostic and consultation processes in healthcare.

Handout: [Quotes from Improving Diagnosis in Healthcare](https://drcachildress-consulting.com/wp-content/uploads/2023/10/Attorney-Improving-Diagnosis-Quotes.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2023/10/Attorney-Improving-Diagnosis-Quotes.pdf>).

Quote Handout: Forensic Custody Evaluations

These are quotes from the [New York Blue Ribbon Commission on Forensic Custody Evaluations](https://ocfs.ny.gov/programs/cwcs/assets/docs/Blue-Ribbon-Commission-Report-2022.pdf) (<https://ocfs.ny.gov/programs/cwcs/assets/docs/Blue-Ribbon-Commission-Report-2022.pdf>), that found them “harmful to children” and as lacking scientific or legal value. Report of the NY Blue Ribbon Commission on Forensic Evaluations

Handout: [NY Blue Ribbon Commission Quotes](https://drcachildress-consulting.com/wp-content/uploads/2023/10/Attorney-NY-Blue-Ribbon-Commission-Quotes.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2023/10/Attorney-NY-Blue-Ribbon-Commission-Quotes.pdf>).

Quote Handout: Psychological Control

These are quotes from the professional literature describing the construct of parental psychological control of the child.

Handout: [Psychological Control Quotes](https://drcachildress-consulting.com/wp-content/uploads/2023/10/attorney-psychological-control-quotes.pdf) (<https://drcachildress-consulting.com/wp-content/uploads/2023/10/attorney-psychological-control-quotes.pdf>).